

FAZIM CAPSULE **(Clofazimine 50mg and 100mg)**

Composition:

Fazim 50mg:

Each capsule contains:

Clofazimine 50mg

Fazim 100mg:

Each capsule contains:

Clofazimine 100mg

Dosage form

Capsule

Product description

50mg & 100mg:

Hard gelatine capsules having scarlet colour cap and body containing Brick Red colour powder, stripped in Alu/pvc blister.

Pharmacodynamics

Clofazimine exerts a slow bactericidal effect on *Mycobacterium leprae* (Hansen's bacillus). Clofazimine inhibits mycobacterial growth and binds preferentially to mycobacterial DNA. Clofazimine also exerts anti-inflammatory properties in controlling erythema nodosum leprosum reactions. However, its precise mechanisms of action are unknown.

Pharmacokinetics

Clofazimine has a variable absorption rate in leprosy patients, ranging from 45%-62% after oral administration. The average serum concentrations in leprosy patients treated with 100 mg and 300 mg daily were 0.7 µg/mL and 1.0 µg/mL, respectively. After ingestion of a single dose of 300 mg, elimination of unchanged Clofazimine and its metabolites in a 24-hour urine collection was negligible. Clofazimine is retained in the human body for a long time. The half-life of Clofazimine following repeated oral doses is estimated to be at least 70 days. Part of the ingested drug recovered from the feces may represent excretion via the bile. A small amount is also eliminated in the sputum, sebum, and sweat.

Clofazimine is highly lipophilic and tends to be deposited predominantly in fatty tissue and in cells of the reticuloendothelial system. It is taken up by macrophages throughout the body. In autopsies performed on leprosy patients, clofazimine crystals were found predominantly in the mesenteric lymph nodes, adrenals, subcutaneous fat, liver, bile, gall bladder, spleen, small intestine, muscles, bones, and skin.

Indication

Clofazimine employed in combination with dapsone and rifampicin, serves as treatment for multibacillary forms of leprosy, such as lepromatous (LL), borderline lepromatous (BL), and midborderline (BB) leprosy (classification according to Ridley and Jopling), as well as erythema nodosum leprosum (ENL).

Combined chemotherapy, e.g., with rifampicin or dapsone, is necessary to prevent the emergence of resistant strains of *M. leprae*.

Recommended dose

For the treatment of leprosy, the WHO recommends the following regimens.

Multibacillary leprosy (LL, BL, BB)

Adults: Clofazimine 300mg once a month under supervision + 50mg once a day as self-medication + rifampicin 600mg once a month under supervision + dapsone 100mg once a day as self-medication.

Children (10-14 years): Clofazimine 200mg once a month under supervision + 50mg on alternate days as self-medication + rifampicin 10mg/kg once a month under supervision + dapsone 1- 2mg/kg once a day as self-medication.

The triple combination should be given for at least 2 years, if possible until skin smears are negative.

Erythema nodosum leprosum (ENL)

Adults and children: If the patient develops ENL, the treatment with rifampicin and dapsone should be continued as before, and the dosage of Clofazimine raised to 200-300 mg daily, given under medical supervision.

These high daily doses must not be given for longer than 3 months.

Method of administration

To ensure maximum absorption Clofazimine should be taken with meals or with milk.

Route of administration

Oral

Contraindication

Hypersensitivity to clofazimine or any component of the formulation.

Warning and precautions

Warnings:

Severe abdominal symptoms (see below) have necessitated exploratory laparotomies in some patients receiving Clofazimine. Rare reports have included splenic infarction, bowel obstruction, and gastrointestinal bleeding. There have also been reports of death following severe abdominal symptoms. Autopsies have revealed crystalline deposits of clofazimine in various tissues including the intestinal mucosa, liver, spleen, and mesenteric lymph nodes.

Clofazimine should be used with caution in patients who have gastrointestinal problems such as abdominal pain and diarrhoea. Dosages of Clofazimine of more than 100 mg daily should be given for as short a period as possible and only under close medical supervision. If a patient complains of colicky or burning pain in the abdomen, nausea, vomiting, or diarrhoea, the dose should be reduced, and if necessary, the interval between doses should be increased, or the drug should be discontinued.

Precautions:**General**

Physicians should be aware that skin discoloration due to Clofazimine may result in depression. Two suicides have been reported in patients receiving Clofazimine.

For skin dryness and ichthyosis, oil can be applied to the skin.

Carcinogenesis, Mutagenesis, Impairment of Fertility

Long-term carcinogenicity studies in animals have not been conducted with Clofazimine. Results of mutagenicity studies (Ames test) were negative. There was some evidence of impaired fertility in one study in rats treated at a dose 25 times the usual human dose; the number of offspring was reduced and there was a lower proportion of implantations.

Interactions with other medicaments**Dapsone:**

Clofazimine seems to have no important effects on the pharmacokinetics of dapsone, although a transient increase in urinary excretion of dapsone occurred in a few patients. Preliminary data suggesting that dapsone inhibits the anti-inflammatory activity of Clofazimine have not been confirmed. If leprosy-associated inflammatory reactions develop in patients being treated with dapsone and Clofazimine, it is still advisable to continue treatment with both drugs.

Rifampicin:

Clofazimine reduces rifampicin absorption in leprosy patients, increasing the time it takes for peak serum concentrations to be reached and prolonging the half-life.

Bioavailability was not affected, so this interaction is unlikely to be clinically significant.

Isoniazid:

In patients receiving high doses of Clofazimine (300mg daily) and isoniazid (300mg daily), elevated concentrations of Clofazimine were detected in plasma and urine, although skin concentrations were found to be lower. Rifampicin accelerates the plasma clearance of dapsone, but this does not necessitate any adjustment of the dosage for the treatment of leprosy.

Pregnancy and lactation

Clofazimine were not teratogenic in laboratory animals at dose levels equivalent to 8 times (rabbit) and 25 times (rat) the usual human daily dose. However, there was evidence of fetotoxicity in the mouse at 12-25 times the human dose, i.e., retardation of fetal skull ossification, increased incidence of abortions and stillbirths, and impaired neonatal survival. The skin and fatty tissue of offspring became discoloured approximately 3 days after birth, which was attributed to the presence of Clofazimine in the maternal milk.

It has been found that Clofazimine crosses the human placenta. The skin of infants born to women who had received the drug during pregnancy was found to be deeply pigmented at birth. No evidence of teratogenicity was found in these infants. There are no adequate and well-

controlled studies in pregnant women. Clofazimine should be used during pregnancy only if the potential benefit justifies the risk to the foetus.

Nursing Mothers

Clofazimine is excreted in the milk of nursing mothers. Clofazimine should not be administered to a nursing woman unless clearly indicated.

Side effects

In general, Clofazimine is well tolerated when administered in dosages no greater than 100 mg daily. The most consistent adverse reactions are usually dose related and are usually reversible when Clofazimine is discontinued.

Adverse Reactions Occurring in More Than 1% of patients

Skin:

Pigmentation from pink to brownish black in 75%-100% of the patients within a few weeks of treatment; ichthyosis and dryness (8%-28%); rash and pruritus (1%-5%).

Gastrointestinal:

Abdominal and epigastric pain, diarrhoea, nausea, vomiting, gastrointestinal intolerance (40%-50%).

Ocular:

Conjunctival and corneal pigmentation due to clofazimine crystal deposits, dryness; burning; itching; irritation.

Other:

Discoloration of urine, feces, sputum, sweats; elevated blood sugar; elevated ESR.

Adverse Reactions Occurring In Less Than 1% of Patients

Skin:

Phototoxicity, erythroderma, acneiform eruptions, monilial cheilosis.

Gastrointestinal:

Bowel obstruction (see WARNINGS), gastrointestinal bleeding (see WARNINGS), anorexia, constipation, weight loss, hepatitis, jaundice, eosinophilic enteritis, enlarged liver.

Ocular:

Diminished vision.

Nervous:

Dizziness, drowsiness, fatigue, headache, giddiness, neuralgia, taste disorder.

Psychiatric:

Depression secondary to skin discoloration; two suicides have been reported.

Laboratory:

Elevated levels of albumin, serum bilirubin, and AST (SGOT); eosinophilia, hypokalaemia.

Other:

Splenic infarction (see WARNINGS), thromboembolism, anaemia, cystitis, bone pain, edema, fever, lymphadenopathy, vascular pain.

Symptoms and treatment of overdose

No specific data are available on the treatment of overdosage with Clofazimine. However, in case of overdose, the stomach should be emptied by inducing vomiting or by gastric lavage, and supportive symptomatic treatment should be employed.

Storage condition

Store in a dry place below 30°C and protect from light.

Shelf life

36 months from date of manufacture

Presentations

Available in blister strip 10 x 10's capsules

Manufactured by:

Lark Laboratories (I) Ltd,
SP-1192 E, Bhiwandi Ind. Area,
Phase –IV, Bhiwandi, Dist. –Alwar,
Rajasthan State, India- 301019.
Reg. off: # A-105/1, Okhla Ind. Area,
Phase-II, New Delhi-110020, India.

Product registration holder:

Zulat Pharmacy Sdn Bhd
23 & 23A, Jln Bandar 3,
Taman Melawati,
53100 Kuala Lumpur,
Wilayah Persekutuan Kuala Lumpur

Date of revision

6th October 2022